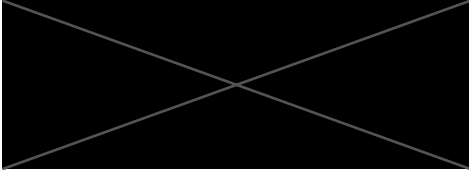
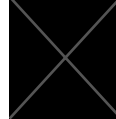


Patient Name:
Patient Number:
Report Date:
Laboratory Use Only



DOB:
Sex:



Page: 1
8/27/2025

Comments:

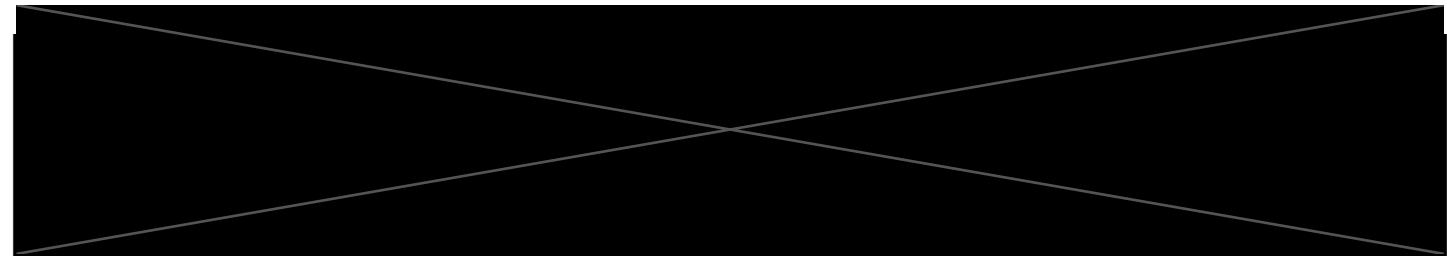
	Result	Value (Previous)	Units	Range	Lab
CBC -Final <i>Ordered by: Avi Retter</i>					
	WBC	4.9 (4.7)	x10 ³ /UL	3.8 - 10.8	ECCC~AV
	RBC	4.82	x10 ⁶ /UL	3.80 - 5.80	ECCC~AV
	Hgb	13.8 (15.0)	g/dl	11.5 - 16.0	ECCC~AV
	HCT	42.7 (46.1)	%	37.0 - 47.0	ECCC~AV
	MCV	88.6	fL	80.0 - 100.0	ECCC~AV
	MCH	28.6	pg	27.0 - 33.0	ECCC~AV
	MCHC	32.3	g/dL	32.0 - 36.0	ECCC~AV
	RDW [Ratio]	15.1	%	11.0 - 16.0	ECCC~AV
	MPV	10.7	fL	6.0 - 12.5	ECCC~AV
	Plat	399 (342)	x10 ³ /UL	150 - 500	ECCC~AV
	Gran%	62.5	%	38.0 - 80.0	ECCC~AV
	Lymph%	22.7	%	15.0 - 51.1	ECCC~AV
	MONO%	9.9	%	0.0 - 13.0	ECCC~AV
	EOS%	4.5	%	0.0 - 8.0	ECCC~AV
	BASO%	0.4	%	0.0 - 2.0	ECCC~AV
	ANC	3.1 (2.9)	x10 ³ /UL	1.5 - 7.8	ECCC~AV
	Lymph#	1.1	x10 ³ /UL	0.9 - 3.9	ECCC~AV
	MONO#	0.5	x10 ³ /UL	0.1 - 1.0	ECCC~AV
	EOS#	0.2	x10 ³ /UL	0.0 - 0.5	ECCC~AV
	BASO#	0.0	x10 ³ /UL	0.0 - 0.2	ECCC~AV

Performing Location: ECCC = Med/Onc - ECCC 2330, 2330 Eastchester Road, #1, , Bronx NY

-- End of Report --

Signed B





Patient Name: [Redacted]
Patient Numb [Redacted]

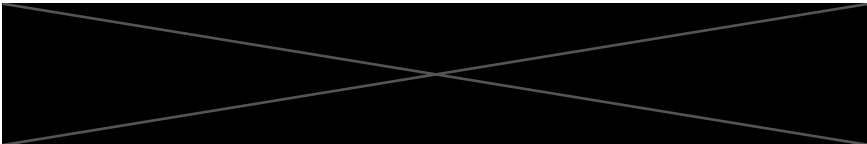
Date: [Redacted]
Date [Redacted]

Diagnosis/Problem

Primary Diagnosis(es): Malignant neoplasm of lower-inner quadrant of left female breast

Date	Type	ICD-9	ICD-10	Description	Disease Status	Status Date
7/3/2025	Primary	174.3	C50.312	Breast Cancer (Breast Cancer) - Clinical Stage IV (AJCC v8) TNM: cT3, cN1, cM1; ER Status: Negative; PR Status: Negative; HER2 (ERBB2) Protein Overexpression (IHC): Negative: 0; HER2 (ERBB2) Gene Amplification (FISH): Negative; Nottingham Grade: G3		
7/3/2025	Addt Codes		Z17.421	[HR-/HER-] HR negative w Hmn epdrml grth fctr recept 2 neg	Active	

Date	Type	ICD-9	ICD-10	Problem	Comment	Status
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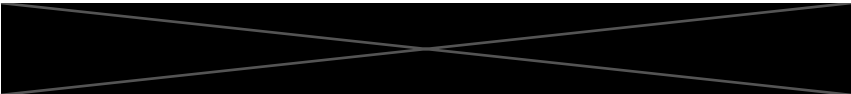


revealed diffusely metastatic disease to the liver and r cancer is not curable and we will not be pursuing ic chemotherapy and try to help her live for as long as d we would add immunotherapy if PDL testing indicated. pies but given her young age and significant burden of

visceral disease I recommend combination therapy. We discussed risks including damage to the organs that it can include heart lung liver kidneys nerves bone marrow. Possible secondary cancers death hospitalization infusion reactions nerve damage infertility nausea vomiting hair loss diarrhea and others. We also gave her drug handouts. Patient was agreeable with plan

genetic testing did not reveal any abnormal mutations

History of Present Illness



ry who in June 2025 boyfriend felt breast abnormality so showed enlarged lymph node as well. These were both uctal carcinoma Ki-67 15%

patient is otherwise completely asymptomatic. She gets normal menstrual cycles though she says they have been slowing down somewhat recently. She absolutely does not want to consider having more children.
Family history -her great aunt had breast cancer
Her maternal cousin had breast cancer
Her paternal grandfather had some kind of cancer as well

No history of cigarette smoking. She drinks occasionally on weekends. She smokes marijuana occasionally



(from surgery note-Baseline diagnostic mammogram/ ultrasound 06/27/2025 showed right breast 12 o'clock 4 cm from the nipple 4 mm cyst and intramammary lymph node, left breast dominant mass at 8 o'clock 5 cm from the nipple measuring at least 1.3 x 2.0 x 1.8 cm highly suspicious with enlarged axillary lymph node 2.2 x 2.6 x 2.5 cm, adjacent smaller abnormal appearing 1 cm lymph node, left breast 2 o'clock 8 cm from the nipple irregular hypoechoic nodule measuring 5 mm likely corresponding to the density on mammogram, BI-RADS 5. Left ultrasound-guided biopsy 07/03/2025 showed left breast to o'clock 8 cm from the nipple benign; left breast 8 o'clock 5 cm from the nipple invasive ductal carcinoma, grade 3, ER/PR/HER2 negative; left axillary lymph node showed similar invasive ductal carcinoma.)

Past medical History

[HR-/HER-] HR negative w Hmn epdrml grth fctr recept 2 neg

Social History

Patient has a significant other.
Past tobacco smoker Not applicable

Family History

Family cancer history: Maternal grandmother with breast cancer diagnosed in her 80s, died at 82. Maternal cousin breast cancer, maternal cousin with stomach cancer.

Personal History

Last Mammogram done:
6/27/25 Last breast ultrasound:
6/27/25

Allergies

No Known Allergies

Medications

Inside	Drug	Script Date	Qty	Rfls	Instructions
Y	Akynzeo (fosnetupitant) 235 mg-0.25 mg/20 mL intravenous solution	8/27/2025	20	7	INFUSE INTRAVENOUSLY 0.25MG EVERY 21 DAYS WITH CHEMOTHERAPY
Y	dexamethasone 4 mg tablet	8/27/2025	20	3	take 2 tabs 24 and 12 hours prior to chemo
Y	ondansetron 8 mg disintegrating tablet	8/27/2025	60	3	1 tab q 8-12 hour prn nausea
	pt reports no meds.	7/30/2025		0	
Y	Udenyca 6 mg/0.6 mL subcutaneous syringe	8/27/2025	0.6	7	INJECT 6MG SUBCUTANEOUSLY EVERY 3 WEEKS POST CHEMOTHERAPY
Y	Udenyca Onbody 6 mg/0.6 mL with wearable subcutaneous injector	8/27/2025	0.6	7	APPLY 6MG ON BODY INJECTOR SUBCUTANEOUSLY EVERY 21 DAYS

Review Of Systems

12 point review of systems with positives reported by patient since last visit.
Nothing reported.
Fever : No Fever. Chills No Rigors. No Chills.

Physical Exam

Normal presentation. In no acute distress. Not anxious. Sclera clear,no jaundice noted. Conjunctiva clear.

Neck: Normal. No thyromegaly. No lymphadenopathy noted.

Cardiovascular: Normal. Regular Rate and Rhythm S1, S2 without murmurs. No Extrasystole.

Chest/Respiratory: Normal. No dyspnea. No rhonchi. No wheezing. No decreased breath sounds at bases. No rales. Unlabored breathing.

No accessory muscle use. Abdomen is soft and nontender to touch. No hepatomegaly noted. No splenomegaly noted. No abdominal pain

or guarding noted. No abdominal mass(es) appreciated. No ascites noted.

Hematologic/Lymphatic: Normal. No petechiae noted. No purpura noted. No lymphadenopathy noted. Normal range of motion in upper

and lower extremities. No edema. No calf tenderness reported by patient

Patient reported pain

Pain Scale 0 - No pain

Treatment Recommendations:

Relaxation

ECOG

Performance Status -ECOG 0: Fully active, able to carry on all pre-disease performance without restriction

Labs

Lab results on 8/19/2025: Glucose=100 mg/dL

Radiology Reports

Print? ☐	Date of Doc.	Name	MD Interpretation	Comment
☐	8/19/2025	PET CT (SKULL BASE TO MID THIGH)		

Pathology Reports

Recommendation/Plan:

breast cancer - stage IV triple negative invasive ductal carcinoma poorly differentiated Ki-67 15% with metastatic disease to the liver lymph nodes and breast

-will plan to treat with carboplatin and Taxol

-would add immunotherapy if PDL testing indicated

- will check Caris on tumor

-will check brain MRI

- will give referral to radiation and palliative care given advanced disease

- genetic testing was negative for BRCA and other mutations

-will ask lab to add on PDL testing to pathology

-will give antiemetics and steroids

Will see patient again in 2 weeks

Current Treatment: Order Activities: MD

Sideeffects were reviewed and appropriate measures of support will be provided.

Patients laboratory results were reviewed and explained in full. All questions answered. Patient's medical records were reviewed.

More than

half of time was spent for providing counseling and coordination of care.

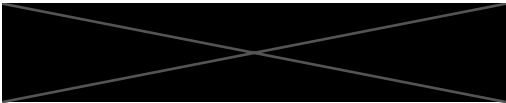
We spent sufficient time to discuss many aspects of care; questions were answered to patient's satisfaction.

The diagnosis and care plan were discussed with patient in detail. Should the patient develop any acute or unexpected medical

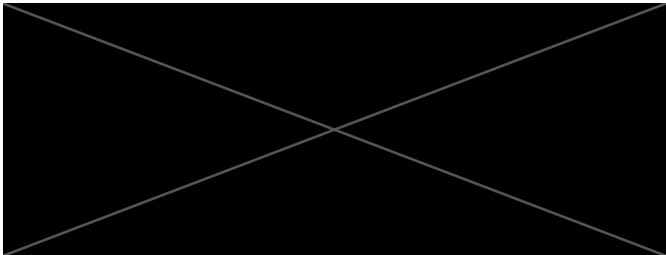
advised to promptly contact our clinic or oncall service if outside of regular office hours. Understanding expressed by patient. Side-effects were reviewed and appropriate measures of support will be provided. We had discussed the composition of chemotherapy regimen, length of chemo-cycle, duration of treatment and the time to assess response to treatment. Order Chemo: . Pathology results reviewed and explained in full. Radiology imaging and dictated report were reviewed; the disease status and level of response to the treatment were explained in great detail. Alternative options were discussed and pros and cons of each option were illustrated. Risk and benefit of the treatment were discussed in detail.
Palliative Care
Radiation Oncology
2 weeks.

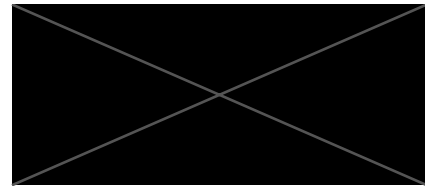
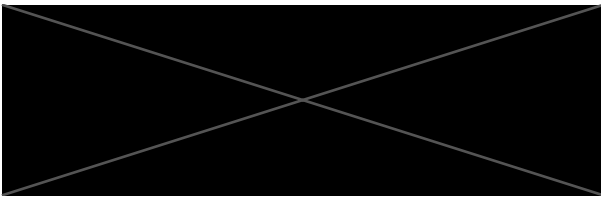
Time spent reviewing the record, seeing the patient and documenting in the medical record:

Time spent reviewing the record, seeing the patient and documenting in the medical record: 45 minutes

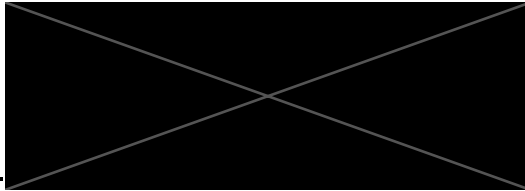


Signe

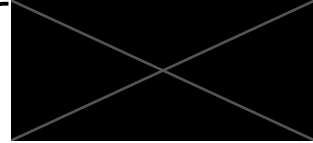




Patient:
DOB / Gender:
MRN:
Location:
Exam Description:



Exam Date:
ACC:
Phys:



[Click for Image Link](#)

FINAL REPORT

HISTORY, REASON FOR EXAM: Breast cancer with positive lymph node lymph node. The patient presents for initial treatment strategy.

TECHNIQUE: Following intravenous injection of 8.1 mCi of F18-FDG, and a delay of 69 minutes, a PET/CT scan from skull base to mid-thighs was acquired. Transmission corrected images were also obtained. The data were reconstructed in coronal, sagittal, transverse and three-dimensional projections. Also, low dose CT scan and fusion images are obtained on this study. The CT protocol used in the PET/CT scan is used for attenuation correction and anatomic localization of PET abnormalities. The companion low dose CT is not designed to produce, and cannot replace state of the art diagnostic CT scans with specific imaging protocols for different body parts and indications. Fingertick glucose of 100 mg/dL.

SCINTIGRAPHIC FINDINGS: No prior PET-CT is available for comparison.

HEAD AND NECK: There is no evidence of FDG avid lymphadenopathy.

CHEST AND MEDIASTINUM: A dominant hypermetabolic nodule in the left breast medially with biopsy clip (2.2 x 1.5 cm; SUV 17.1: Series 2, image 131). Additional smaller nodular foci of activity in the left breast (SUV up to 4). Multiple FDG avid nodes in the left axillary and left subpectoral nodal stations. For example, a left axillary node with biopsy clip (3.2 x 2.5 cm; SUV 22.5: Series 2, image 99) and a left subpectoral node (1.9 x 1.3 cm; SUV 18.4: Series 2, image 89). There is no abnormal uptake in the lungs.

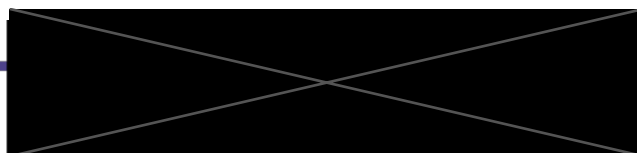
ABDOMEN AND PELVIS: Extensive foci involving both hepatic lobes in a background of hypodensities, for example, one in the left hepatic lobe (4.5 x 2.4 cm; SUV 11.8: Series 2, image 157). Increased activity associated with nodes below the diaphragm. For example a node medial to the liver (1.5 x 1 cm; SUV 4.6: Series 2, image 177) and a periportal node (2.3 x 1.4 cm; SUV 5.2: Series 2, image 197). Enlarged uterus with mild heterogeneous activity (13.3 x 10 cm; SUV up to 3.4: Series 2, image 285).

MUSCULOSKELETAL: There are no FDG avid osseous foci suggestive of malignant involvement.

Low dose CT demonstrates: Nonavid pulmonary nodular densities, for example, a cavitory nodule in the right upper lobe (1.1 cm (series 2, image 82), a 0.7 cm nodule in the right upper lobe (series 2, image 85) and a 0.5 cm pleural-based nodule in the left upper lobe (series 2, image 114); subcentimeter nonobstructing left renal calculus; scoliosis with orthopedic hardware in the thoracolumbar spine.

IMPRESSION:

1. Hypermetabolic foci involving the left breast, left axillary/left subpectoral nodal stations, liver and nodes below the diaphragm compatible with malignancy and metastatic disease.
2. Enlarged uterus with mild heterogeneous activity. Consider correlation with pelvic ultrasound.
2. Nonavid pulmonary nodular densities. Attention to follow-up.



Patient:



MRN / DOB:



Thank you for the opportunity to participate in the care of this patient.

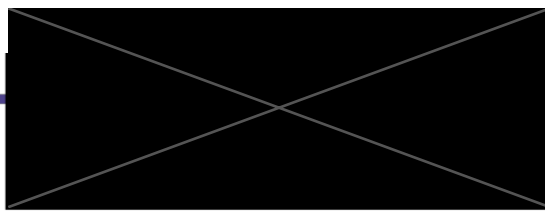
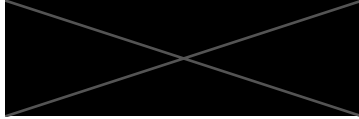
Physician direct line:

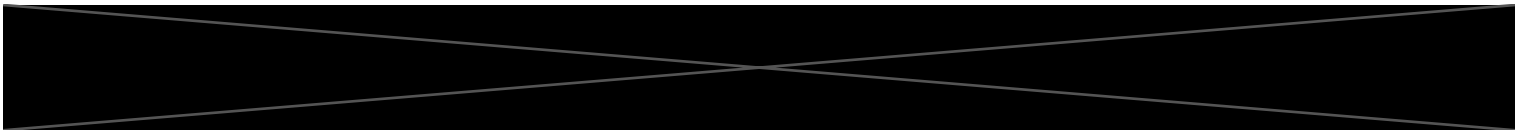


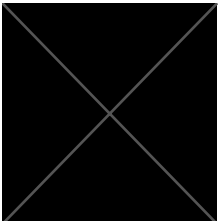
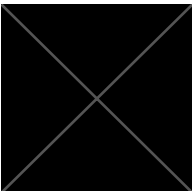
Electronically Signed By:

Sign Date: 08/19/2025 10:

Interpreting Radiologist:





Patient name: DOB: Sex assigned at birth: Gender: Patient ID (MRN):		Sample type: Sample collection date: Sample accession date:		Report date: Invitae #: Clinical team:	
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Reason for testing

Diagnostic test for both a personal and a family history of disease

Test performed

Sequence analysis and deletion/duplication testing of the 48 genes listed in the Genes Analyzed section.

- Invitae Common Hereditary Cancers Panel



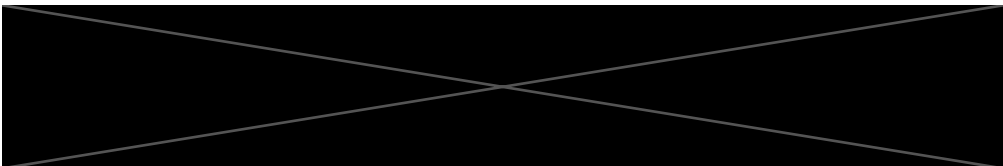
RESULT: NEGATIVE

About this test

This diagnostic test evaluates 48 gene(s) for variants (genetic changes) that are associated with genetic disorders. Diagnostic genetic testing, when combined with family history and other medical results, may provide information to clarify individual risk, support a clinical diagnosis, and assist with the development of a personalized treatment and management strategy.

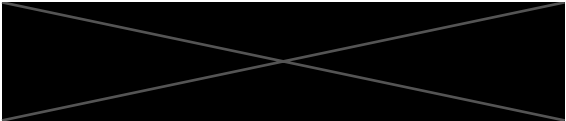
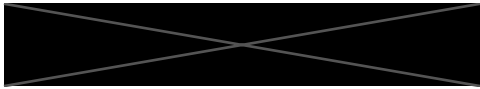
Next steps

- This test did not identify any pathogenic variants known to cause disease. This result does NOT exclude a genetic diagnosis and should be discussed with a healthcare provider, such as a genetic counselor, to learn about the appropriate next steps for further evaluation. Clinical follow up may still be warranted. This result should be interpreted within the context of additional laboratory results, family history and clinical findings.
- Register your test at www.invitae.com/patients to download a digital copy of your results. You can also access educational resources about how your results can help inform your health.



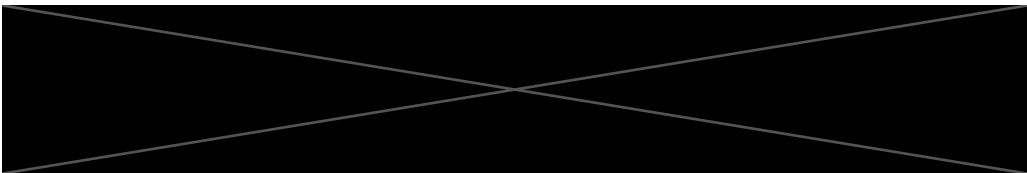
INVITAE DIAGNOSTIC TESTING RESULTS

Patient n
Invitae #:



Clinical summary

No reportable genetic variants were identified by this analysis, however, this individual may still be at risk for certain medical conditions based on other factors such as family history, genetic causes not evaluated with this test, or other environmental influences. Follow up of this individual and surveillance of their family members may still be indicated.



INVITAE DIAGNOSTIC TESTING RESULTS

Patient
Invitae

Genes analyzed

This table represents a complete list of genes analyzed for this individual, including the relevant gene transcript(s). If more than one transcript is listed for a single gene, variants were reported using the first transcript listed unless otherwise indicated in the report. An asterisk (*) indicates that this gene has a limitation. Please see the Limitations section for details. Results are negative unless otherwise indicated in the report. Benign and Likely Benign variants are not included in this report and in specific scenarios variants of uncertain significance in the requisitioned gene(s) may not be included in this report.

GENE	TRANSCRIPT
APC*	NM_000038.5
ATM*	NM_000051.3
AXIN2	NM_004655.3
BAP1	NM_004656.3
BARD1	NM_000465.3
BMPRI1A	NM_004329.2
BRCA1	NM_007294.3
BRCA2	NM_000059.3
BRIP1	NM_032043.2
CDH1	NM_004360.3
CDK4	NM_000075.3
CDKN2A (p14ARF)	NM_058195.3
CDKN2A (p16INK4a)	NM_000077.4
CHEK2	NM_007194.3
CTNNA1	NM_001903.3
DICER1*	NM_177438.2
EPCAM*	NM_002354.2
FH*	NM_000143.3
GREM1*	NM_013372.6
HOXB13	NM_006361.5
KIT	NM_000222.2
MBD4	NM_003925.2
MEN1*	NM_130799.2
MLH1*	NM_000249.3
MSH2*	NM_000251.2
MSH3*	NM_002439.4
MSH6*	NM_000179.2
MUTYH	NM_001128425.1
NF1*	NM_000267.3
NTHL1	NM_002528.6
PALB2	NM_024675.3
PDGFRA	NM_006206.4
PMS2*	NM_000535.5
POLD1*	NM_002691.3
POLE	NM_006231.3

GENE	TRANSCRIPT
PTEN*	NM_000314.4
RAD51C	NM_058216.2
RAD51D	NM_002878.3
SDHA*	NM_004168.3
SDHB	NM_003000.2
SDHC*	NM_003001.3
SDHD	NM_003002.3
SMAD4	NM_005359.5
SMARCA4	NM_001128849.1
STK11	NM_000455.4
TP53	NM_000546.5
TSC1*	NM_000368.4
TSC2	NM_000548.3
VHL	NM_000551.3

INVITAE DIAGNOSTIC TESTING RESULTS

Patient name
Invitae #: RQ

Methods

- Genomic DNA obtained from the submitted sample is enriched for targeted regions using a hybridization-based protocol, and sequenced using Illumina technology. Unless otherwise indicated, all targeted regions are sequenced with $\geq 50\times$ depth or are supplemented with additional analysis. Reads are aligned to a reference sequence (GRCh37), and sequence changes are identified and interpreted in the context of a single clinically relevant transcript, indicated in the Genes Analyzed table. Enrichment and analysis focus on the coding sequence of the indicated transcripts, 20bp of flanking intronic sequence, and other specific genomic regions demonstrated to be causative of disease at the time of assay design. Promoters, untranslated regions, and other non-coding regions are not otherwise interrogated. For some genes only targeted loci are analyzed. Exonic deletions and duplications are called using an in-house algorithm that determines copy number at each target by comparing the read depth for each target in the proband sequence with both mean read-depth and read-depth distribution, obtained from a set of clinical samples. Markers across the X and Y chromosomes are analyzed for quality control purposes and may detect deviations from the expected sex chromosome complement. Such deviations may be included in the report in accordance with internal guidelines. Variants are reported according to the Human Genome Variation Society (HGVS) guidelines. Confirmation of the presence and location of reportable variants is performed as needed based on stringent criteria using one of several validated orthogonal approaches (PubMed ID 30610921). Sequencing is performed by Labcorp Genetics, Inc (1400 16th Street, San Francisco, CA 94103, #05D2040778). Confirmatory sequencing is performed by Labcorp Genetics, Inc (1400 16th Street, San Francisco, CA 94103, #05D2040778). RNA sequencing is performed by Labcorp Genetics, Inc (1400 16th Street, San Francisco, CA 94103, #05D2040778).

The following additional analyses are performed if relevant to the requisition. For PMS2 exons 12-15, the reference genome has been modified to force all sequence reads derived from PMS2 and the PMS2CL pseudogene to align to PMS2, and variant calling algorithms are modified to support an expectation of 4 alleles. If a rare SNP or indel variant is identified by this method, both PMS2 and the PMS2CL pseudogene are amplified by long-range PCR and the location of the variant is determined by Pacific Biosciences (PacBio) SMRT sequencing of the relevant exon in both long-range amplicons. If a CNV is identified, MLPA or MLPA-seq is run to confirm the variant. If confirmed, both PMS2 and PMS2CL are amplified by long-range PCR, and the identity of the fixed differences between PMS2 and PMS2CL are sequenced by PacBio from the long-range amplicon to disambiguate the location of the CNV. For C9orf72 repeat expansion testing, hexanucleotide repeat units are detected by repeat-primed PCR (RP-PCR) with fluorescently labeled primers followed by capillary electrophoresis. Interpretation Reference Ranges: Benign (Normal Range): <25 repeat units, Uncertain: 25-30 repeat units, Pathogenic (Full Mutation): ≥ 31 repeat units (PMID: 21944779, 22406228, 23111906, 28689190, 31315673, 33168078, 33575483). A second round of RP-PCR utilizing a non-overlapping set of primers is used to confirm the initial call in the case of suspected allele sizes of 22 or more repeats. For RNA analysis of the genes indicated in the Genes Analyzed table, complementary DNA is synthesized by reverse transcription from RNA derived from a blood specimen and enriched for specific gene sequences using capture hybridization. After high-throughput sequencing using Illumina technology, the output reads are aligned to a reference sequence (genome build GRCh37; custom derivative of the RefSeq transcriptome) to identify the locations of exon junctions through the detection of split reads. The relative usage of exon junctions in a test specimen is assessed quantitatively and compared to the usage seen in control specimens. Abnormal exon junction usage is evaluated as evidence in the Sherlock variant interpretation framework. If an abnormal splicing pattern is predicted based on a DNA variant outside the typical reportable range, as described above, the presence of the variant is confirmed by targeted DNA sequencing.

- A PMID is a unique identifier referring to a published, scientific paper. Search by PMID at <http://www.ncbi.nlm.nih.gov/pubmed>.
- An rsID is a unique identifier referring to a single genomic position, and is used to associate population frequency information with sequence changes at that position. Reported population frequencies are derived from a number of public sites that aggregate data from large-scale population sequencing projects, including ExAC, gnomAD, and dbSNP.
- A MedGen ID is a unique identifier referring to an article in MedGen, NCBI's centralized database of information about genetic disorders and phenotypes. Search by MedGen ID at <http://www.ncbi.nlm.nih.gov/medgen>. An OMIM number is a unique identifier referring to a comprehensive entry in Online Mendelian Inheritance in Man (OMIM). Search by OMIM number at <http://omim.org/>.
- The laboratory uses information from individuals undergoing testing to inform variant interpretation. If "internal data" is cited as a reference in the variant details this may refer to the individual in this requisition and/or historical internal observations.

Limitations

Based on validation study results, this assay achieves $>99\%$ analytical sensitivity and specificity for single nucleotide variants, insertions and deletions $<15\text{bp}$ in length, and exon-level deletions and duplications. This test detects insertions and deletions larger than 15bp but smaller than a full exon but

INVITAE DIAGNOSTIC TESTING RESULTS

Patient name
Invitae #: RQ

DOB:

sensitivity for these may be marginally reduced. Deletion/duplication analysis for this test determines copy number at a single exon resolution at virtually all targeted exons. However, in rare situations, single-exon copy number events may not be analyzed due to inherent sequence properties or isolated reduction in data quality. Certain types of variants, such as structural rearrangements (e.g. inversions, gene conversion events, translocations, etc.) or variants embedded in sequence with complex architecture (e.g. short tandem repeats or segmental duplications), may not be detected. Additionally, it may not be possible to fully resolve certain details about variants, such as mosaicism, phasing, or mapping ambiguity. Unless explicitly guaranteed, sequence changes in the promoter, non-coding exons, and other non-coding regions are not covered by this assay. Please consult the test definition on our website for details regarding regions or types of variants that are covered or excluded for this test. This report reflects the analysis of an extracted genomic DNA sample. While this test is intended to reflect the analysis of extracted genomic DNA from a referred patient, in very rare cases the analyzed DNA may not represent that individual's constitutional genome, such as in the case of a circulating hematolymphoid neoplasm, bone marrow transplant, blood transfusion, chimerism, culture artifact or maternal cell contamination. Interpretations are made on the assumption that any clinical information provided, including specimen identity, is accurate. RNA analysis is not designed for use as a stand-alone diagnostic method and cannot determine absolute RNA levels. Results from the RNA analysis may not be informative for interpreting copy number events. Additionally, sensitivity to detect RNA splicing events may be reduced for variants in the first donor site of each gene and for complex rearrangements or events.

APC: Sequencing analysis for exons 5 includes only cds +/- 10 bp. ATM: Sequencing analysis for exons 6, 24, 43 includes only cds +/- 10 bp. DICER1: Sequencing analysis for exons 22 includes only cds +/- 10 bp. EPCAM: Sequencing analysis is not offered for this gene. FH: Sequencing analysis for exons 9 includes only cds +/- 10 bp. GREM1: Promoter region duplication testing only. MEN1: Sequencing analysis for exons 2 includes only cds +/- 10 bp. MLH1: Sequencing analysis for exons 12 includes only cds +/- 10 bp. MSH2: Analysis includes the exon 1-7 inversion (Boland mutation). Sequencing analysis for exons 2, 5 includes only cds +/- 10 bp. Deletions restricted to only the EPCAM gene will not be detected unless EPCAM analysis is requested. MSH6: Sequencing analysis for exons 7, 10 includes only cds +/- 10 bp. NF1: Sequencing analysis for exons 2, 7, 25, 41, 48 includes only cds +/- 10 bp. PMS2: Sequencing analysis for exons 7 includes only cds +/- 10 bp. POLD1: Sequencing analysis for exon 22 includes only cds +/- 10 bp and exon 27 includes only cds +/- 0 bp. PTEN: Sequencing analysis for exons 8 includes only cds +/- 10 bp. SDHA: Deletion/duplication analysis is not offered for this gene and sequencing analysis is not offered for exon 14. Sequencing analysis for exons 6-8 includes only cds +/- 10 bp. SDHC: Sequencing analysis for exons 2, 6 includes only cds +/- 10 bp. TSC1: Sequencing analysis for exons 21 includes only cds +/- 10 bp. MSH3: Sequencing analysis of the repeat region of exon 1 (5:79950697-79950765) is not offered.

Disclaimer

DNA studies do not constitute a definitive test for the selected condition(s) in all individuals. It should be realized that there are possible sources of error. Errors can result from trace contamination, rare technical errors, rare genetic variants that interfere with analysis, recent scientific developments, and alternative classification systems. This test should be one of many aspects used by the healthcare provider to help with a diagnosis and treatment plan, but it is not a diagnosis itself. This test was developed and its performance characteristics determined by Labcorp Genetics. It has not been cleared or approved by the FDA. The laboratory is regulated under the Clinical Laboratory Improvement Amendments (CLIA) as qualified to perform high-complexity clinical tests (CLIA ID: 05D2040778). This test is used for clinical purposes. It should not be regarded as investigational or for research.

This report has been released utilizing a validated procedure approved by:

GENERAL GUIDELINES NEGATIVE RESULTS GUIDE

This document is not part of the Invitae® clinical report and does not represent medical advice. These are general guidelines that are not specific to your result and may not represent all relevant international recommendations. You can use this guide to talk to your healthcare provider about your test results, clinical history, and the most current guidelines. We recognize that individuals have diverse gender and sexual identities. In this guide, the terms female, male, women, and men refer to sex assigned at birth.

What is a negative result?



A negative test result means that no significant genetic changes (“pathogenic” or “likely pathogenic” variants) were found. Risk for disease can still be influenced by a combination of personal, lifestyle, environmental, and/or unidentified genetic factors.

Create a plan with a healthcare provider



It is important to share these results with a healthcare provider to determine appropriate next steps. The chance for an individual to develop a disease is not usually determined by genetic test results alone.

What does this result mean for family members?



Parents, siblings, children, and other relatives have their own genetic makeup. Although these results did not find a significant genetic change, family members can discuss their own potential health and/or reproductive risks and the option of genetic testing with their own healthcare providers.

Resources



Genetic counseling can help individuals understand their genetic test results and options for next steps. Reviewing test results with a genetic counselor or other healthcare provider is recommended. Local or telehealth genetic counselors can be identified using the Find a Genetic Counselor search tool at nsgc.org (US and Canada). Individuals with an Invitae test result can also log in to their patient portal (invitae.com) to view their results, contact a genetic counselor, or join Invitae’s Patient Insights Network (PIN) (pin.invitae.com), an online platform where individuals can share information about their health and experiences to help advance research and drug development.